The Asylum

The Opal: A Monthly Periodical of the State Lunatic Asylum, Devoted to Usefulness, Edited by the Patients of the Utica State Lunatic Asylum

(1850–1860)

Beginning in November 1850, the patients at the Utica State Lunatic Asylum in upstate New York began writing, editing, and publishing a monthly newsletter, the *Opal*. Dedicating their effort to "usefulness," patients and ex-patients were given remarkable license to pen essays, poems, and reflections. Proceeds from subscriptions were used to stock the patient library collection. The monthly was eventually discontinued in 1860. Since the *Opal* was directed at readers both inside and outside the asylum, its stories and articles provide a glimpse into not only how patients viewed insanity and themselves, but also how they perceived staff, treatment, and the outside world.

Truthfulness with the Insane (1852)

The most numerous by far of all cases of conscience brought constantly into the casuistical court are those which relate to the duties of Truth. "Are

William Hogarth (1697–1764), *Scene in Bedlam* (1735), from *A Rake's Progress* (plate 8). Bedlam, or Bethlem (medieval variations of "Bethlehem"), Hospital in London began specializing in the care of the insane in 1403. Well into the nineteenth century, it was possible for locals and travelers to tour madhouses, much like the two women portrayed here. Etching and engraving on paper. Tate Gallery, London, Great Britain. © Tate, London/Art Resource, NY.

we bound to speak the truth at all times?" Who has not, at some periods of his experience, been perplexed with this question, and longed to know in what way to resolve it. . . .

Now, we venture to say, there is no one case of conscience more commonly deemed easy of solution, than that which has regard to the duty of *truthfulness with the sick and the insane*. We put both of these classes of persons together, because, for our purpose, the question is substantially the same with respect to both. An insane man is a man under the influence, commonly, of some bodily weakness or disease, and it is a very common effect of bodily sickness to produce, in greater or lesser degree, mental derangement.

We say there is no case of conscience more easily solved, according to the popular estimation, than this one with reference to truth-telling

towards the sick and insane. In fact, it has become hardly a question at all, with the great majority. Leaving the insane out of the question, who does not know how common is the practice of equivocation and deceit towards the sick? Who does not know how often *physicians* lead the way in this sort of dealing? It is not an unfrequent thing—we speak from our own observation—that physicians conduct themselves in this particular, as if they were absolved from all obligation to the rules of veracity by virtue of their profession. How often does it occur that they flatter their patients with speedy, or, at least, ultimate recovery, when they have already judged the case to be hopeless, and the sufferer is already lying upon the verge of the grave. We have known the sick, and the family that watched around the bed-side, kept in utter ignorance of the true state of the progress of the disease, at the same time that the patient was rapidly sinking into the arms of death, and the physician who had spoken only words of assurance and hope in his ear, was telling to all *without* the household that recovery was impossible! We trust that such a course is not characteristic of the profession in general. We are glad to know that by the best medical authorities, and by our most scientific and distinguished practitioners, it is entirely disapproved. We hesitate not, to brand such conduct on the part of the appointed guardians of the sick-bed, to be as uncalled for and cruel as it is treacherous and wicked.

There are many who will assent to the justness of the views stated above, who will very likely dissent from us when we come to speak of the expediency and duty of *truthfulness towards the insane*. There are multitudes who, doubtless, consider it neither expedient nor a duty to observe strict veracity with this unfortunate class of persons. It is, we believe, a general impression that those who have to do with the insane in our Asylums are governed by no rules upon the subject, unless it be the rule of employing both truth and falsehood, according as one or the other shall be best suited to the particular exigency. This, in fact, was our own impression, until our residence and entrance as a patient within the walls of the N.Y. State Asylum gave us an opportunity to ascertain the policy actually adopted. . . . It will, perhaps, be presuming too much, but we will venture one or two suggestions upon this subject. . . .

First then, Insanity is but another word for delusion, the *delusion* of falsehood, and falsehood manifestly, therefore, is not the proper cure for a disease of which it is, in itself, the essence. The thing which needs to be

expelled from the mind of an insane person, before it can be recovered to soundness, is the falsehood under which it labors, and how this can be best done by injecting new falsehood into the mind, we may well be at a loss to know. If a child has been fed upon sweetmeats until it has become pale and thin like a skeleton, is it best, in order to its recovery, to continue the sweetmeats or to endeavor to neutralize and overcome their already hurtful effect by a new and nourishing diet? There is but one answer to this question, an answer suggested by the very nature of the case, as we should say. Now as with the emaciated body, so with the deranged mind. No mind can be in a health condition that feeds upon falsehood. It must, of course, be diseased and hastening to decay. What it needs is something different and opposite to that which it has fed upon, and that something is TRUTH.

It is certain, furthermore, that the insane are more or less susceptible to all influences exerted in consistency with the requirements of veracity. It no uncommon thing in the experience of those who have charge of institutions for the insane to find the delusion of their patients giving way before a continual representation of the truth. Nor is it unfrequent that the recovered patient is able to call to mind, how the truthful declarations of his physician first broke in upon his delusions with persuasive power, and how from this source the first ray of light shone upon the brooding darkness. We can speak from experience, how much it contributed to the rest of a mind tossed upon the billows of phrenzy and despair, when we had gained the conviction that those who were placed in charge of us were men in whose slightest utterance we could have confidence; men who made it a sacred principle not to deceive their patients in any particular. There is a certain point of recovery when the disordered mind seeks to discriminate between that which is true and that which is false in its condition. It has delusions which it would gladly rid itself of, if it might dare to do so; but it has others, equally hurtful, to which it would as gladly hold fast. Then also, there are some of which are pleasant, and others, it may be disagreeable to contemplate; but now, as he finds his delusions beginning to dissipate, what shall he do with these? Ought he to retain his confidence in them while he dismisses it in the others? In such circumstances what can be more grateful to the tempest-tost soul than to have at hand a faithful counsellor, whose every word is truth, and who may be relied on to guide his trembling steps through the maze he is treading.

Editor's Table (1852)

Considerable anxiety is sometimes expressed by persons who derive a morbid satisfaction from looking on scenes of human misery, as to the propriety, safety, &c., of their visiting the Asylum. This diseased state of the sentiments is most incident to those who have been badly educated, and who, especially, have not been taught to follow up feeling by the corresponding actions. They are mightily stirred by a story of distress, but never think of an effort to relieve it. The natural tie between emotion and conduct has suffered a violent disruption.—We do not like to see such people passing through the halls. The authoress of the following letter belongs to the class we have been describing, and a decent respect for her sex demands that we should not pass it by unnoticed.

> To the Editor of The Opal:
>
> Sir,— My father is a citizen of the State of New York, and a voter in regular standing, and he has told me that by reason of these things he and his family have free admission to the Asylum. As I have not much to do at home, (mother and the help doing all the work), I proposed a few days ago that we should have a good sleigh-ride and fetch up at the Asylum. We were a merry party. When we got up on the big stoop and among the stone pillars, we were surprised to have a man say to us, that "it was past the hour for visitors." We were indignant of course, and told him that we had come to go through the Asylum, and not to learn the time of day. He said that he would have to speak to some one; and soon a man came to us, (a mighty handsome one by the way) and fixing a great searching black eye upon us, said to us that we might go through; but he scared us all by cautioning us with a tone and look, which I shall never forget, to bear in mind that we were in a Hospital for the Insane. So we went around. Every thing was as clean as clean can be. (I hope mother wont go there; for she is forever dinging at me about dirt in my room): We were all disappointed. For all we could see, the patients look and act like other people. We asked our guide, who was civil enough, if he wouldn't take us where we could see *something*. He politely bowed us away to the sleigh.
>
> Now, Sir, if this is all that one can see in your place, I beg leave to tell you that I shall not visit it again, and shall dissuade all my friends from doing so.

This is all at present from
Yours to command,
Araminta C. Stubbs
P.S.—Pray, is not the young man who went round with us a bit of a wag, or is he one of the patients? A.C. S.

Miss Araminta C. complains that "for all she could see, the patients look and act like other people." Ah—could she look into the inner soul of those whose apparent composure has so disappointed her sickly and vulgar anticipations! Could she see the heart aching with a grief which will not and cannot be comforted—or withered by long and solitary indulgence in thoughts of the neglect or scorn of the world, which, whether real or imaginary, cannot be removed by the sympathising tones nor cheering smiles of that love which always soothes and animates a mind in trouble—or torn and racked by passions which are always contending with each other, and, having no reality for their object, may never give any outward manifestation of the agonizing tumult which reigns within!

But Araminta complains, that she was not taken where she could "see something."—What does she mean by something? Is it slam-bang, kick, rear up, smash windows, make fun, and yell? Such things are to be seen outside of, as well as within the Asylum; but however entertaining they might be to the lady, the performance of them might hazard her safety. There are, we own, many queer cases amongst us, whose idiosyncrasies, while they are very funny, never endanger the safety of the most timid or the most pure. There is especially one little fellow whose ideas run incessantly on osculation. An uncontrollable desire to kiss every thing he sees is his failing. And the dreadful looks of the objects he sometimes selects for this pastime are the chief grounds for declaring him insane. . . .

Editor's Table (1855)

"An Hour with the Insane," says the courteous editor of the N.Y. *Commercial Advertiser*, "we spent the other day." Not that the editor was really here, in *propria persona*, but in perusing the pages of the OPAL, his mind and kind sympathies were here. Well, others, we trust, have in like manner, "spent an instructive hour," and others, still, might find the perusal of its unpretending pages instructive and pleasing, were they to receive it. The

fragrance of the rose is not diminished by its seclusion from the embellished floral bower—nor are the thoughts of truth and beauty which find utterance here, less fraught with the aroma of wisdom because they emanate from the seclusive quite of Asylumia.

If not our gratitude, certainly our pride is awakened by the fraternal etiquette of the *Advertiser*—somewhat critical though it is, since by having nearly now reached its half century, it deserves, as it surely has, our need of veneration—and the manner of its allusion to our opalescent gems is in nowise unappreciated. But we deem it proper to say, here, that its articles are all written by patients, and under no other "supervision" or restraint than their own *genii*. The beloved and honored Superintendent, nor either of his estimable Assistants interpose any control or direction in the productions of the brains or pens of the contributors to the OPAL—that is, they do not advise or supervise in the matter, farther than to express their decisions that such and such individuals, thus desiring—and many are *here*, somehow seized with *author-mania*, who before never thought of the thing—may be furnished with writing materials and opportunities to "improve their gifts." Every one thus furnished, writes "what is written;" and as written, each article appears, with such alterations only, as the principal editor, or the printer, might properly make in similar cases, regarding punctuation, &c. And when we reflect that many of our most approved articles are written by patients occupying halls where there is little to superinduce that elasticity of mind, and that gaiety of feeling indicated by their compositions—but where, on the contrary, they are often unavoidably subjected to annoyances from insane singing, talking, laughing, weeping, walking, etc, etc.—scenes ever to be found among a company of insane persons—we think we have reason to feel a degree of satisfaction with, if not literary pride of our productions. And what encouragement doth this one thought of itself afford the Superintendent and his Assistants, that they do not "labor in vain," in a most humane and important department of society, while laboring to restore their unfortunate fellow-beings to health, usefulness and happiness.

And what higher incentive can be presented to the enlightened legislator or to the statesman, in favor of a liberal appropriation of the public funds for supplying the Institution with all needful facilities for the care and restoration to active usefulness the men and women who, for various rea-

sons, it becomes necessary, for a time, to isolate from the associations and responsibilities of life at large, in the halls of the Asylum?

Views of Insanity by an Ex-patient (1859)

As far as any personal experience goes, we are rather at a loss to convey to the mind of the sane reader a definite idea of insanity. We were going to say we never had a spectral vision; but a perhaps Quixotic desire to stick to the truth, leads us to record the following incident: when a youngster we once fell asleep on the kitchen floor: being suddenly aroused to consciousness, we for but an instant of time—hardly that, fancied we saw a *gray cat* in the very act of falling upon us: this was our first and last apparition. During a period antecedent to the last ten years our physical frame was subject to some disturbances; therefore in giving any opinion as to the essence of insanity—excepting what we make *inferentially* from witnessing or hearing of diseased mentality in others, we draw upon our memory of what we were at that period.

Without pretending to a very accurate knowledge of metaphysical or medical science, our knowledge enables us to make these two classifications of insanity—diseased will; diseased perception. Under the first class, we are forced to enroll ourselves; in the second, not at all; yet, ten years ago, we might have properly been under that other classification.

Previous to the time in question, *we dreamed* (perhaps in a secondary sense the observation applied to the day as well as the night): *during* that time our slumbers have been dreamless. To this assertion we add the following qualification: we have sometimes awakened from an apparently unbroken rest and been told that we had talked in our sleep. Nevertheless we believe that during the period specified, unbroken rest has been the rule, and dreaming the momentary exception.

Let us without making open war against the dogmas of science, in making our *classification*, record our *impression* that the first has (in a great measure) but an existence in the imagination of men—that the proper nomenclature of moral insanity is wicked will. Acting under the conviction to a great extent, we state our opinions. *Insanity is the stuff that dreams are made of.* We fancy it closely allied to somnambulism. In some temperaments, under severe exertion of the mental and physical powers, the mind is withdrawn from contact with surrounding objects, while the body

continues mechanically to perform its customary actions—to exemplify which, a canal-driver, I think, told me that he had, when on foot, following his horse in the night-time, fallen asleep, yet continuing to follow his horse. We mention also that after the toil of a summer's day (during the period when our physical frame was not what it is now) playing on an instrument of music, we have fallen asleep and awakened, without, as we believed, disturbing the progress of the music.

Insanity, dreams, clairvoyance, somnambulism, and catalepsy—all, we have an opinion, are links between the terrestrial and spirit world. This last is the region or regions where the spirit, disembodied, betakes itself after death—and, under abnormal conditions, before. In these abnormal states the mind has *gone*, partially or wholly. Where a slight degree of exterior (physical) perception remains, an insane person seems to have his physical senses, seeing, etc.,—while he is in most respects like one in a fit or cataleptic trance. That insanity is but a waking-day-sleep, is gathered not from our own experience, but that of lunatics with whom we conversed after they had become sound.

From *Opal: A Monthly Periodical of the State Lunatic Asylum, Devoted to Usefulness, Edited by the Patients of the Utica State Lunatic Asylum* (Utica, NY: Utica State Lunatic Asylum), 1851–1860; excerpts are from the following: 1852, vol. 2, 33–35, 121–122; 1855, vol. 5, 188–189; 1859, vol. 9, 32–33.

Limerick District Lunatic Asylum

Report of the Limerick District Lunatic Asylum for the Year Ending December 31st, 1866

(1867)

Superintendents of asylums were generally obligated to submit an annual report to those organizations and authorities in charge of supporting and supervising the facility. These reports provide fairly detailed information about matters such as the number of admissions and discharges, patient behavior and treatment, staff, and the status of buildings and grounds. The following is an excerpt from an annual report submitted by the physician Robert Fitzgerald, resident medical superintendent of the Limerick District Lunatic Asylum in Ireland, to local political and church authorities. What appear at first glance to be rather mundane statistical tables reveal a great deal about the kinds of individuals institutionalized, how life in the asylum was organized, and how administrators categorized patients and disorders. Note that the abbreviation "Do." below stands for "ditto."

Return of the Number of Patients Admitted, Discharged, Died, and Escaped within the Year

	Males	Females	TOTAL
Number of patients remaining in Asylum on 31 December 1865	224	219	443
Do. Patients admitted to 31 Dec 1866	31	39	70
TOTAL	255	258	513
Number Discharged Recovered during the year . . .	20	17	
Do. Improved	4	10	
Do. Unimproved	2	3	
Do. Escaped	1	0	
Do. Deaths	2	9	
TOTAL			68
Remaining in Asylum on 31 Dec 1866	226	219	445
Distribution by Counties			
City of Limerick	21	31	52
County of Limerick	110	103	213
County of Clare	95	85	180
TOTAL	226	219	445
State as to Probability of Recovery of those in Asylum on 31 December 1866			
Lunatics Probably Curable	49	46	95
Lunatics Probably Incurable	154	149	303
Lunatics, Idiots	9	10	19
Lunatics, Epileptics	14	14	28
TOTAL	226	219	445
Classification of Patients according to Physician's Abstract of Cases			
Mania—Probably Curable	49	46	95
Do. Do. Incurable	154	149	303
Do. Idiots	9	10	19
Do. Epileptics	14	14	28
TOTAL	226	219	445
Daily Average Number of Patients			443

Daily Average Number of Patients Employed during the Year Ended 31st December, 1866

Male		Female	
Garden Labour	2	Needle Work	35
Agricultural Labour	30	Knitting	34
Tailoring Work	1	Assisting in Laundry	18
Carpentry	1	Cleaning House	18
Cleaning House	14	Miscellaneous	23
Miscellaneous	8		
TOTAL EMPLOYED	**56**	TOTAL EMPLOYED	**128**

Work Done by Female Patients

Made: 400 shirts, 240 sheets, 364 chemises, 180 gowns, 162 petticoats, 310 caps, 498 aprons, 89 bed ticks, 124 bolsters, 24 table cloths, 12 bath towels, 13 rollers, 604 pairs of stockings, and 623 pairs of socks.

Repaired: 500 jackets, 601 pairs of trousers, 276 waist coats, 571 shirts, 244 sheets, 167 blankets, 134 bed ticks, 149 bolsters, 800 gowns, 921 chemises, 621 petticoats, 214 caps, 281 aprons, 10 table cloths, 47 shawls, 3814 pairs of stockings, and 4124 pairs of socks.

Authority for Admissions

Authority for Admission of Patients, and Number Admitted during the Year ended 31st December, 1866

	Males	Females	TOTAL
Ordinary cases admitted by order of the Board	4	7	11
Ordinary cases admitted as urgent by the Physicians	16	23	39
Dangerous Lunatics, by Warrant of the Lord Lieutenant	9	6	15
Lunatics charged with offences, by Warrant of the Lord Lieutenant	2	3	5
TOTAL	31	39	70

Age of Patients

Age of Patients Admitted and Discharged Recovered during the Year ended 31st December, 1866

	Admitted Male	Admitted Female	Admitted TOTAL	Discherged Cured Male	Discharged Cured Female	Discharged Cured TOTAL
Under 10	1	1	2	0	1	1
10–20	17	4	21	0	2	2
20–30	5	8	13	11	8	19
30–40	6	9	15	3	2	5
40–50	0	11	11	4	3	7
50–60	0	5	5	0	1	1
60–70	1	1	2	1	0	1
70 and upwards	1	0	1	1	0	1
TOTAL	31	39	70	20	17	37

Duration of Illness

Duration of Disease previous to Admission in those Discharged Recovered during the Year ended 31st December, 1866

	Males	Females	TOTAL
Under 1 month	5	3	8
Under 3 months	3	1	4
Under 6 months	2	1	3
Under 9 months	1	1	2
Under 1 year	2	1	3
Under 2 years	1	1	2
Under 3 years		2	2
Under 4 years		2	2
Under 5 years	1	2	3
Under 6 years			
Under 8 years			
Under 10 years and upwards			
Unknown	5	3	8
TOTAL	20	17	37

Length of Stay

Length of Residence in Asylum of those Discharged, Recovered, and Improved during the Year ended 31st December, 1866

	Recovered Male	Recovered Female	Recovered TOTAL	Improved Male	Improved Female	Improved TOTAL
Under 2 months	7	2	9	2	1	3
Under 4 months	2	2	4			
Under 6 months	2	2	4		1	1
Under 8 months		2	2			
Under 12 months	2	6	8	1	3	4
Under 18 months	3		3			
Under 2 years	1	2	3	1		1
Under 3 years	3	1	4	1		1
Under 4 years					1	1
Under 5 years					1	1
Under 6 years					1	1
Under 10 years					2	2
TOTAL	20	17	37	5	10	15

Form of Illness (Admittees)

Form of Disease in those Admitted
during the Year ended 31st December, 1866

	Male	Female	TOTAL
Acute Mania	15	20	35
Chronic Mania	13	12	25
Melancholia		1	1
Melancholia Religious	1	2	3
Hereditary	2	3	5
Imbecility		1	1
TOTAL	31	39	70

Form of Illness (Those Remaining)

Form of Disease in those Remaining in Asylum on 31st December, 1866

	Male	Female	TOTAL
Mania Acute and Chronic	137	111	248
Melancholia Religious	39	47	86
Dementia	26	35	61
Monomania	1		1
Imbecility		2	2
Idiocy	9	10	19
Mental Affection	14	14	28
TOTAL	226	219	445

Education

Educational Condition of Patients in Asylum on 31st December, 1866

	Male	Female	TOTAL
Well Educated	21	14	35
Can Read and Write Well	24	28	52
Can Read and Write Indifferently	70	37	107
Can Read Only	48	49	97
Cannot Read or Write	50	81	131
Unknown	13	10	23
TOTAL	226	219	445

Marital Status

Social Condition of Patients in Asylum on 31st December, 1866

	Male	Female	TOTAL
Married	35	57	92
Single	187	130	317
Widowers and Widows	4	26	30
Unknown		6	6
TOTAL	226	219	445

Conduct

Classification of Patients in Asylum on 31st December, 1866

	Male	Female	TOTAL
Convalescent	18	9	27
Quiet and Orderly, but Insane	90	73	163
Moderately Tranquil	70	58	128
Noisy and Refractory	48	79	127
Imbecile and Epileptic	14	16	30
Suicidal		4	4
TOTAL	226	219	445

Previous Occupation

Previous Occupation of those in Asylum 31st December, 1866

	Male	Female	TOTAL
Labouring Class	145	44	189
Farming	9		9
Domestic Servants	1	38	39
Clerks	2		2
Shopkeepers	5	4	9
Tailors and Seamstresses	4	5	9
Artisans	2		2
Painters and Glaziers	1		1
Smiths and Workers in Metals	1		1
Masons and Bricklayers	1		1
Carpenters	4		4
Shoemakers	3		3
Hatters	1		1
Factory Workers	1		1
Vicutallers	3		3
Pedlars and Hucksters	1	5	6
Lawyers			
Medical Men	2		2
Members of Religious Communities	1		1
Students and Teachers	5	2	7
Soldiers and Pensioners	10		10
Police	2		2
Revenue Officers	2		2
Sailors	1		1
Publicans	1		1
Various Employments			
TOTAL	208	98	306
No Occupation or Unknown	18	121	139
TOTAL	226	219	445

Causes of Death

Causes of Death during the Year ended 31st December, 1866

	Male	Female	TOTAL
Debility		3	3
Congestion of Brain		2	2
Senile Insanity	1	1	2
Marasmus		1	1
Consumption		1	1
Exhaustion from Epilepsy	1	1	2
TOTAL	2	9	11

Escapes and Escape Attempts

Male	Female	TOTAL	Observations
	1	1	Attempted to get away but was caught inside the walls
1		1	Climbed over the front gate but was caught immediately
1		1	Attempted to get out through the Window of his Cell at Night
1		1	Effected his Escape, and his Friends kept him at home, where he ultimately became quite well

Supposed Cause of Mental Illness

Supposed Cause of Mental Disease of Patients
in Asylum on 31st December, 1866

	Males	Females	TOTAL
MORAL CAUSES			
Poverty and Reverse of Fortune	13	15	28
Grief, Fear, and Anxiety	15	12	27
Love, Jealousy, and Seduction	3	9	12
Domestic Quarrels and Afflictions	2	4	6
Religious Excitement	5	8	13
Study and Mental Excitement	5	3	8
Ill-Treatment		7	7
Pride		1	1
Anger		1	1
Kleptomania	1	1	2
TOTAL MORAL CAUSES	44	61	105
PHYSICAL CAUSES			
Intemperance and Irregularity of Life	10	5	15
Cerebral Diseases or Affections	24	21	45
Congenital Idiocy, Etc.		1	1
Febrile Affections		9	9
Effects of Climate and Sunstroke	5	1	6
Bodily Injuries and Disorders	7		7
Abuse of Medicine	2		2
Sedentary Habits	3	3	6
TOTAL PHYSICAL CAUSES	51	40	91
Hereditary	29	28	57
Not Known	102	90	192
TOTAL	226	219	445

Patients under Restraints

Number of Patients Placed under Mechanical Restraint or Seclusion
during the Year ended 31st December, 1866

Male: 0

Female: 14 (all with application of straight Waistcoat)

Francisco José de Goya y Lucientes, *Lunatic Asylum* (ca. 1812–1819). It is known that Goya visited an asylum around the area of Sargasso. The Sargasso asylum was part of a hospital complex first built in 1425, and, similar to what Goya depicts here, patients at the asylum were given tent cloth to wear, violent patients were kept in cells of wood or limestone, and the floor was covered with straw. The Sargasso asylum was also noteworthy, however, in that the inmates put on regular theatrical performances for visitors. Canvas. Real Academia de Bellas Artes de San Fernando, Madrid, Spain. © Erich Lessing/ Art Resource, NY.

Amusements, Games, Books, &c.

Description of Amusement and Games	Class of Books and Periodical supplied to Patients	Male	Female	TOTAL
Ball playing	Newspapers and periodicals	24		**24**
Card playing	"	36	36	**72**
Foot Ball	"	50		**50**
Instrumental Music	"	6		**6**
Music and Dancing	"	20	40	**60**

Patients' Dietary

Meals	Dietary	Male	Female	TOTAL
ORDINARY				
Breakfast	7 oz. of Indian Meal and Cutlins made into Stirabout	155	90	**245**
	12 oz. of Bread for Males and 8 oz. for Females	71	129	**200**
	1 pint of New Milk	200	120	**320**
	1 pint of Tea	26	99	**125**
Dinner	12 oz. of Bread, or a 1/4 stone of Potatoes for Males, 8 oz. of Bread or 3 lbs. of Potatoes for Females	226	219	**445**
	12 oz. of Beef on Sundays and Thursdays, 8 oz. on Mondays and Tuesdays, Ox Head soup on Saturdays,1 pint of Milk or Coffee on Wednesays and Fridays	219	213	**432**
Supper	6 oz. of Bread and 1/2 pint of Milk	226	219	**445**
	Tea		6	**6**
EXTRA				
Breakfast	Butter			
Dinner	Roast Beef and Mutton			
HOSPITAL DIET				
	Porter, Wine, Whiskey, Rice, and Brandy			

From Limerick District Lunatic Asylum, *Report of the Limerick District Lunatic Asylum for the Year Ending December 31st, 1866* (Limerick: G. M'Kern & Sons, 1867).

Office of Superintendent Government, Great Britain

Annual Report of the Insane Asylums in Bengal for the Year 1867

(1868)

Lunatic asylums first appeared in India toward the end of the eighteenth century. What began as an attempt by the East India Company to build replicas of British asylums on Indian soil was maintained by the British Crown after it formally established its rule in 1858. The spread of British asylums throughout India was justified on the same humanitarian grounds touted earlier by the likes of Pinel, Esquirol, and Tuke: a colonial obligation to advance and civilize backward people and institutions.

Since British colonials and authorities considered themselves superior to their colonized subjects, however, they were unwilling to compel British nationals to share facilities with native Indians. Thus, a segregated system was established. Insane Indians were confined in shabby, public institutions, while their European counterparts were accommodated in a private asylum, given time to recuperate, and then returned to their homeland. The following annual report describes conditions in the public asylum in Patna.

From Surgeon R. F. Hutchinson, M.D. (Superintendent, Patna Lunatic Asylum) to Surgeon-Major H. M. Macpherson (Secretary, Inspector General, Medical Department, Lower Provinces). Patna, 1 January 1868

Sir,

I have the honor of submitting the usual Asylum Report for the year 1867, and before entering upon the history of the year, I would draw special attention to the following objects of importance: —I. The necessity for increased accommodation in the Asylum buildings; and

II. The necessity of adapting the present erection to the present advanced condition of sanitary sciences.

But then the question may be put, *in limine*, why interfere with either? the Asylum has jogged on very well in the years that are past; let it do so equally in the years to come. I think, however, that, with the improvements I am about to suggest, it will be allowed that matters would progress still more favourably: that the present rate of sickness and mortality would be still further diminished, and the general comfort and happiness of the inmates still better maintained.

I. In May 1866, I reported on the crowded state of the Asylum, and suggested a mode of relief. It was then overcrowded with 138 patients. How much more so it is now with 151! The general form and proportions of the Asylum are known to the authorities; so I shall merely observe that the main building has accommodation for 59 insanes; the old Civil Jail holds 25; and the female side has 20 cells. Thus the maximum accommodation is for 104 patients, not taking into consideration the deep verandahs in both Asylums; but with cell accommodation for 104 patients, we have a total of 151 (113 males and 38 females), or 47 beyond our capacity. Such being the case, (and we are receiving weekly additions to our strength) some means should be devised for relieving the pressure before a dire epidemic of cholera sweeps away the surplus, as it did in 1866.

II. The sanitary improvements necessary. The drainage everywhere is excellent; but then the building is situated on low ground, which every year is subject to inundation. Last year the flood was so high that all the drains were full of backwater, and in 1861 the water was knee-deep at the south end of the Asylum enclosure. The buildings have not yet recovered from the effects of last year's flood; so great was the saturation of the soil, and the attendant percolation, that the mud floors of the manufacturing sheds, though a foot above the ground, were puddles, and the foot holes of the looms were little wells.

The south end of the main building was untenable from the damp which showed on the walls three feet from the floor; and from having a northern aspect, and never being reached by the sun, it remains damp and disagreeable, and unfit for occupation.

The sickness might have been considerable, had it not been for the large quantities of straw which I laid in for the insanes to lie upon.

This evil cannot, of course, be remedied without either raising the plinth or removing the Asylum bodily to a higher site.

The ventilation of the main block of the Asylum is very defective, and necessarily so from its shape, which, being that of a large quadrangle running North and South, can only receive ventilation from the prevailing eastern and western winds. Therefore the north and south ranges of cells rarely, if even, have a breath of air through them, as their doors and windows are all north and south, and they have dead walls to the East and west. These remarks only apply to the main block, the rest of the Asylum being admirably ventilated.

To increase the accommodation, and throw open the whole building to air and sunshine, I would suggest the following alterations, illustrating them by a plan of the main block. Remove entirely the northern and two southern rows of cells, and extend to the northern the eastern and western rows of cells. This extension would exactly compensate for the removals, and we should thus have two long rows of airy and well ventilated cells, accommodating 60 patients; and from there being no enclosing walls, there would be less risk of damp. To the north of the old Dewani Jail, I would erect two parallel blocks for 40 patients, and in the same manner extend the female block to the south, allowing accommodation for 40 patients, instead of 20, as at present. Thus the Asylum would be capable of separately accommodating 165 patients as follows:

Main block	60 patients
New block	40 patients
Dewani block	25 patients
Female block	40 patients
Total	165 patients

Our present strength being 150.

IV. On the whole, the health of the Asylum has been very good, considering the risk and discomfort to which the insanes were exposed by the inundation. The mortality was greater than in the previous year, but then our numbers were considerably increased; so that the balance is rather in favor of 1867.

As usual, dysentery and diarrhea carried off the majority of the fatal cases, there being ten cases of both. Of the nine dysentery cases, not one was struck down in robust health; four were cases of old age; and the other five were admitted feeble and emaciated.

V. The conservancy of the Asylum has been carefully attended to, the dry-earth system being enforced as strictly as possible. I have largely used McDougall's Disinfecting Powder, and find it very valuable as a deodoriser. Until the inundation, the cells were regularly leeped, and are so now, except in the south end of the male Asylum, where the damp still prevails so greatly.*

VI. The food of the insane has, on the whole, been good. Tricks have at times been played with the ata; and to prevent their occurrence, I am anxious to grind our own flour on the premises.†

VII. The clothing has been ample, and of good quality, every part of it manufactured in the Asylum; and its regular washing has been strictly attended to. But we have suffered a good deal at the hands of unruly patients, male and female, whole thans of dosotee having been torn into shreds, and numerous blankets consigned to the same fate.‡

VIII. The manufactures are carried on steadily within certain limits. Insanes will work steadily in the beaten paths; but it is rather difficult to lead them across country by teaching them arts entirely new for they are by no means sharp in picking up knowledge, and Return N. 6 [data not included here] will show that we have not a very large stock of workmen to draw upon.

However, we manage, in considerable degree, to pay our way, and have a nice balance to our credit as well.

IX. I have repeatedly had occasion to draw attention to the unsatisfactory, if not illegal, manner in which alleged insanes are sent to the Asylum.

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*Editor's note: Leeping involves washing something with cow dung and water.

†Editor's note: *Ata* is whole wheat flour typically used to make flatbread.

‡Editor's note: *Thans* refers to lengths of cloth; *dosotee* refers to a particular type of fabric.

In many instances patients have been sent in with merely a scrap of vernacular writing; in others no proper descriptive roll is sent; or if it is, the information is most meagre, and at times contradictory. Rarely, if ever, is the strict order requiring personal examination by a Magistrate carried out, and the consequence is that many a man, reeling about the bazaar intoxicated with ganjah or spirit, finds himself, on coming to his senses, an inmate of a Lunatic Asylum.*

And this incarceration, however temporary, is by no means a trifling matter; for let the man ever thereafter religiously eschew ganjah or spirit, he will never remove the stigma from his name that he once was págul, and once an inmate of a págul-khána.†

Return No. 4 [data not shown] shows how large a proportion of the admissions is due to ganjah and bháng.‡ Under the seduction and maddening influence of these poisons, so openly sold and easily procured, many a career, opening hopefully and prosperously, has terminated in sorrow and gloom. How bitterly the first, and perhaps only, whiff of that deadly chelum is lamented; how hopelessly the first, and perhaps solitary, draught of bháng is deplored!**

We must give the native credit for having feelings like ourselves. We must allow him a conscience which, though it be frequently dormant, is still open to the chidings of remorse.

And it is a sad, though interesting, study to watch the workings of the mind in many a poor patient. The reply with difficulty extracted,—the face hung down and averted,—the love of silence and solitude,—all indicate the smitings of conscience and gnawings of remorse, and excite a sincere pity for the unhappy victim to such seductions.

I have now in the Asylum two or three such cases; one in particular, where the patient, a Bengali lad, seems overwhelmed with a sense of his disgrace and degradation.

X. While noticing one undoubtedly powerful agent in creating insanity, we may fitly examine the pretensions of another supposed to be equally

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*Editor's note: *Ganjah* is also known as cannabis and hashish.

†Editor's note: *Págul* means "insane" or "mad"; a *págul*-khána is an insane asylum or madhouse.

‡Editor's note: A preparation from cannabis.

**Editor's note: *Chelum* here refers to hashish.

powerful in exciting it. We may try and ascertain whether the word "lunacy" has any real claims to the derivation assigned to it. Anxious to test the reality or otherwise of lunar influence on the insane, I carried out during the year the following observations, adding to them notes of the pressure, temperature, and humidity of the atmosphere.

As they only extend over a year, they are perhaps not very valuable, but still the results are interesting; the more so, as they are, I believe, at variance with prevailing ideas. Thus, extremes of temperature are not necessarily accompanied by maniacal exacerbation, for the greatest heat, 110° F, was noted on June 4th and September 10th, and the greatest cold, 40°, on December 15th, and on neither day was any patient excited. Nor are extremes of pressure; for the Barometer was at its utmost height, 30–33, on November 23rd at its greatest depression, 29–31, on July 7th, without any cases of maniacal excitement.

Humidity apparently predisposes to excitement; for on the day of greatest rain, July 29th (when four inches were gauged), one patient was attacked; and in July, the month of greatest rain, (when _____ were gauged),* we had the maximum excitation; in the rainy months of June, July, August, September, and October we had 45.5 percent of all cases of excitement.

I should fancy that the mugginess of the rains has as much to say to these cases as the positive humidity. Now for lunar influence, or lunacy proper. The annexed Table will show that the lunations had but little influence on the patients attacked; the greatest number falling to the share of the new moon, and smallest but one to that of the full moon. And that the full moon has apparently but little influence will be seen from this Table (B), which shows that, while forty-nine cases of excitement occurred during the wax, eighty-nine occurred during the wane of the moon.

On the whole, I think the question is still an open one, to be decided by further experiment; and I believe that magnetic and electric observations will yield valuable results. These I hope to commence on the receipt of the instruments I have sent home for.

XI. I regret that I can only repeat the remarks in my Report for 1865 regarding the Asylum establishment. As a class, the keepers are of little

*Editor's note: The author left this blank.

Interior view of a corridor in the asylum at Norristown, Pennsylvania, United States (1896). Courtesy of the National Library of Medicine, Bethesda, MD.

worth, and I have not received from Sergeant Frawley, the Overseer, the assistance I anticipated. He is greatly wanting in activity, and has not the confidence or respect of his subordinates.

XII. I append a list of the visits paid to the Asylum during the year by the Official Visitors:

Jan	None		
Feb	Officiating Deputy Inspector General		
Mar	Deputy Inspector General; Judge; Joint Magistrate		
Apr	Deputy Inspector General		
May	Ditto	Ditto	
June	Ditto	Ditto	
July	Ditto	Ditto	
Aug	Ditto	Ditto	
Sep	Ditto	Ditto	
Oct	Ditto	Ditto	Commissioner
Nov	Ditto	Ditto	
Dec	None		

I have the honor to be, Sir,
Your most obedient Servant,
R. F. Hutchinson, M.D.
Superintendent, Lunatic Asylum

From W. A. Green (Inspector General of Hospitals, Lower Provinces, Office of Superintendent Government), *Annual Report of the Insane Asylums in Bengal for the Year 1867* (Calcutta: Office of Superintendent Government Printing, 1868), 71–75.

Elizabeth P. W. Packard

(1816–1897)

The Prisoners' Hidden Life, or Insane Asylums Unveiled

(1868)

Already in the eighteenth century, there were widely publicized reports about sane individuals being committed against their will as well as stories about the abuse of patients in asylums. By the second half of the nineteenth century, novels and short stories, such as Sheridan Le Fanu's *Uncle Silas* (1864) and Charlotte Perkins Gilman's *The Yellow Wallpaper* (1899), were replete with tales of madwomen kept in attics and of husbands or guardians attempting to drive their wards mad. Such horror stories were reinforced throughout the century by former patients who alleged they were mistreated in asylums. Their memoirs often had sensational titles, for example, Samuel Bruckshaw's *One More Proof of the Iniquitous Abuse of Private Madhouses* (1774) and Elizabeth Packard's *The Prisoners' Hidden Life, or Insane Asylums Unveiled* (1868). By the late nineteenth century, these reports helped lend credence to calls for a far-reaching reform of state hospitals.

The following is an excerpt from Elizabeth Packard's *The Prisoners' Hidden Life*. A champion for married women's rights, Packard

accused her husband of committing her to the Jacksonville Insane Asylum in Illinois against her will. She claimed that her husband, in collusion with the asylum's director, kept her there for espousing Calvinist religious beliefs. Eventually gaining her freedom, she published a number of books in the years 1864–1868 about her experiences, going on to form one of the first antipsychiatry associations in the United States. Even in the absence of knowing the veracity of her claims, her account below shows how conventional ideals of femininity and family inflected early antipsychiatric sentiments.

Mrs. Cheneworth's Suicide—Medical Abuse

Mrs. Cheneworth hung herself in her own room, after retiring from the dancing party, last night. Her measure of grace was not sufficient to enable her to bear the accumulated burdens of her hard fate any longer, without driving her to desperation. I can not blame her for deliberately preferring death, to such a life as she has been experiencing in this Asylum. She has literally been driven to it by abuse.

She was entered in my ward, where she remained for several weeks, when she was removed to the lowest ward, where she has been murdered by slow tortures. If this Institution is not responsible for the life of Mrs. Cheneworth, then I don't know what murder is. She was evidently insane when she entered; she was not responsible, although her reason was not entirely dethroned. Her moral nature was keenly sensitive; her power of self-control was crushed by disease and medical maltreatment. She resisted until she evidently saw it was useless to expect justice, and was just crushed beneath this powerful despotism.

She was a lovely woman, fitted by nature and education to be an ornament to society and her family. Gentle and confiding, with a high sense of honor and self-respect, she despised all degrading associations. . . . She was a most accomplished dancer, having been trained in the school of the best French dancers in the country. Her complexion white and clear, with regular features, black, but mild and tender eyes, her hair was long, black, and beautiful. In short, she was a little, beautiful, fawn-like creature, when she came to this Institution. She had been here a short time once before, after the birth of her first child; and from her account I inferred

that her restoration to reason was not then attended with the grim spectre of horrors which must have inevitably accompanied this.

She had left a young babe, this time, which her physician advised her to wean, since she was now in a delicate condition. Thus her overtasked physical nature, abused as it was by bad medical treatment, added to the double burden she was called to endure, could not sustain the balance of her mental faculties. Her nerves were unstrung, and lost their natural tone by the influence of opium, that most deadly foe of nature, which evidently caused her insanity. The opium was expected to operate as a quietus to her then excited nervous system; but instead of this, it only increased her nervous irritability. The amount was then increased, and this course persisted, until her system became drunk, as it were, by its influence. The effect produced was like that of excessive drinking, when it causes delirium tremens. Thus she became a victim to that absurd practice of the medical profession, which depends upon poisons instead of nature to cure disease.

What Mrs. Cheneworth wanted was, the nourishment of her exhausted physical nature, by rest, food, air, and exercise. She did not need to have the power of her system thrown into confusion by taxing them with poisons, which nature must either counteract and resist, or be overcome by them, and sink into death. Nature was importuning for help to bear her burdens, being already overtasked. But instead of listening to these demands, her blinded friends allowed her to be thus medically abused. After having suffered her to receive this treatment, and thus brought into a still worse condition—an insane state—when more than ever she needed help and the most tender, watchful care; then to be cast off in her helplessness upon strangers, who knew nothing of her character, her habits, her propensities, her cravings, her disposition, or her constitution; how could they reasonably expect her to thus receive the care necessary to her recovery? They probably did expect it, and on this false expectation placed her here for appropriate medical treatment.

What a delusion the world is laboring under, to expect such treatment here! Did they but know the truth, they would find that all the "medical treatment" they get here is to lock them up! and thus having hidden them from observation, and cut them off from all communication with their friends, they then inflict upon them what they consider condign punishment for being insane! Why can not their friends bestow upon them this "medical treatment" at home, without the expense of sending them to this

Asylum to get it? This is the sum and substance of all the "treatment" they get here, which they could not get at home—that is, they could not get *this treatment* from reasonable friends, any where, outside of these inquisitorial institutions. How doleful is this purgatory? thus legally upheld for the *punishment of the innocent*! Great God! Is this Institution located within the province of thy just government? or is this Satan's seat, that has not yet been subjected to thy omnipotent power? . . .

Alas! for poor Mrs. Cheneworth! her days for reasonable treatment expired when she was removed to the lowest ward, and consigned to the care of Elizabeth Bonner. This attendant was a perfect contrast to her former attendants in character, disposition, and habits. She was a large, coarse, stout, Irish woman, stronger than most men; of quick temper, very easily thrown off its balance, when, for the time being, she would be a perfect demon, lost to all traces of humanity. Her manners were very coarse and masculine, a loud and boisterous talker, and a great liar, with no education, and could neither read nor write.

To this vile ignorant woman was Mrs. Cheneworth entrusted, to treat her just as her own feelings dictated. Miss Bonner's first object was to "subdue her," that is, to break down her aspiring feelings, and bring her into a state of cringing submission to her dictation. Here was a contest between her naturally refined instincts, and Miss Bonner's unrefined and coarse nature. Any manifestation of the lady-like nature of Mrs. Cheneworth, was met by its opposite in Miss Bonner's servant-like nature and position, and she must lord it over this gentle lady. The position of the latter, as a boarder, must at her beck, be exchanged, by her being made to feel that she was nothing but a slave and menial. If she ventured to remonstrate against this wanton usurpation of authority over her, she could only expect to receive physical abuse, such as she was poorly able to bear. And O! the black tale of wrongs and cruel tortures this tender woman experienced at the hand of this giant like tyrant no tongue or pen can ever describe! She was choked, pounded, kicked, and plunged under water, and well nigh strangled to death. Mrs. Coe assured me this was only a specimen of the kind of treatment all were liable to receive at her hands, since she claimed that this was the way to cure them! and this insisted upon, was what she was put here to do. Being strong, she was peculiarly adapted to her place, since no woman or man could grapple with her successfully.

The last time I saw Mrs. Cheneworth was at the dance, after which she hung herself, being found suspended from the upper part of her window by the facing of her dress. I never saw a person so changed. I did not know her when Miss Bonner introduced me to her that evening. O, such a haggard look! such despair and wretchedness as her countenance reflected, I have never witnessed. My feelings were touched. I asked her to go with me, and putting my arm around her waist, she walked with me across the ward to the window looking South. Here we conversed confidentially, freely. She said, "O, Mrs. Packard, I have suffered everything but death since we were parted!"

"But how has your face become so disfigured by sores, and what causes your eyes to be so inflamed?"

"I fainted, and fell down stairs, and they poured camphor so profusely over my face, and into my eyes and ears, that I have, in consequence, been blind and deaf for some time."

I do not know whether her chin, which was red and raw, was thus caused or not. She said the fall had caused her to miscarry, and thus, thought I, you have had to bear this burden in addition to the load of sorrows already heaped upon your tender, weak person. Said I, "Have you any hope of getting out of this place—of ever being taken to your friends?"

"No! none at all! Hopeless, endless torment is all that is before me! O, if I could only get out of this place, I would walk to my father's house. It is only fourteen miles south here," pointing out the window, "but O, these iron bars! I can not escape through them." . . .

Here we leave Mrs. Cheneworth, and turn with sorrowing hearts, to the group of bereaved ones at home—those fondly loved ones, who have thus been called to lay upon the altar of sacrifice, this precious victim. O, could you have foreseen her sad fate, would you thus willingly have laid her upon such an altar? No, you would not. You could not, and lay claim to your humanity. You are not hard hearted and cruel towards this loved idol of your fondest affections. No, you would have cherished her with the tenderest care at home, had you thought it would have promoted her best good. Your hearts, I doubt not, wept the bitterest tears at the thought of being compelled to place her in an Insane Asylum. But these tears could not remove the necessity which you felt you had for so doing. Had you not reason in your own mind for believing that Insane Asylums were established for the benefit of the insane? Did you not suppose they had a com-

petent medical faculty there, who knew better than yourselves how to treat such cases? Yes, so you thought, as you ought to have had reason to think.

But alas! for a blinded public! Alas! for man who is placed under an irresponsible human power. Such power, man is not fitted to be trusted with. Despotism too soon usurps the rule of reason and kindness, and might takes the place of right. Authority supplants kindness, truth, and honesty. After this love of domineering has once taken possession of the human soul, it can only be held by sinister, artful policy. Helplessness, weakness, and dependence are the virgin soil where tyranny and despotism hold their most resistless sway. But under the influence of our free government, power would probably cope with it successfully; therefore its policy consists in cutting off these victims from access to any power by which they would be exposed and dethroned. Therefore, they not only prevent communications with their friends while there, but forestall their confidence in their statements after they get out, assuring them they were so insane while there that they can not report correctly, and therefore their representations must be listened to as mere phantoms of a diseased imagination. Therefore, their friends hear as though they heard not.

But the hitherto blinded public can no longer plead ignorance as an excuse for not grappling successfully with this legalized despotism. No; the Legislature of the State are already informed, through their own Committee, of the imperative need of such enactments, as shall hereafter forever prevent such *abuse of power*, by any future Superintendent, as their present incumbent is found to be notoriously guilty of.

From Mrs. E.P.W. Packard, *The Prisoners' Hidden Life; or, Insane Asylums Unveiled* (Chicago: E.P.W. Packard and A. B. Case, 1868), 202–211.